

# The content library

Eight hours of recording. Three sites, five settings. Around fifty pieces, made for Lagos.

PREPARED FOR

Dr Chinwe Kpaduwa and Dr Itunu Akinware

BY

Consult for Africa

STATUS

For discussion. Nothing here is settled, and the shot list is deliberately longer than the day.



Dear Chinwe and Itunu,

This is not a call sheet. It is the document we would like to argue with you about for two hours, after which somebody can write a call sheet.

We have read a great deal of what Chinwe has already made, and the short version is that the thesis is finished. The position is clear, it is unusual, and it is hers: that ageing is not a problem to be solved, that the best work is the work nobody can point at, and that knowing when not to operate matters more than any menu. We are not going to spend eight hours re-recording that.

What the existing library cannot do is convert somebody in Ikoyi, for one plain reason. It was made for an American audience and it never mentions Nigeria. The woman weighing this up here is not asking what an abdominoplasty is. She is asking who will be holding the instrument, what happens at two in the morning if something goes wrong, whether she will look obviously done, and whether she should simply fly to Istanbul instead.

Those questions have Lagos answers. Nobody has filmed them. That is the whole opportunity, and it is why this is worth a serious day rather than a content sprint.

Three things we would like from the session. Tell us where our read of the audience is wrong, because you have sat across from more of these women than we have. Tell us which of the provocations in section four are good and which are embarrassing. And bring the questions you are tired of being asked, because those are almost always the ones worth filming.

Debo Odulana

Consult for Africa

## 1. What we think you already believe

We have set this out so you can correct it rather than build it from scratch. If any of it is wrong, the shot list is wrong too.

| What your own work says                                            | Where it shows up                                                                     |
|--------------------------------------------------------------------|---------------------------------------------------------------------------------------|
| <b>Ageing is not the problem people think it is</b>                | A recurring argument across your videos, and the sharpest thing you say               |
| <b>The best work is invisible, and obviousness is a failure</b>    | Your most consistent position, and the origin of the fireside title                   |
| <b>Knowing when not to operate matters more than the menu</b>      | Your refusal pieces, which are the most distinctive content you have                  |
| <b>Darker skin is not lighter skin with more pigment</b>           | Your skin-of-colour material, and the single most under-served subject in this market |
| <b>The industry manufactures insecurity and should be resisted</b> | Your anti-influencer and business-of-insecurity pieces                                |
| <b>A doctor is not a content creator</b>                           | Stated plainly, more than once                                                        |

That last one matters for this shoot. We are not asking you to become an influencer, and the plan below is built so that you never have to sell anything. You explain, the clinic books. That is also, conveniently, the only configuration Nigerian regulation permits.

## 2. The six things that actually stop a booking



This is our read of the Lagos buyer. Argue with it.

| The blocker                    | What she is actually asking                              | What answers it                                                                                    |
|--------------------------------|----------------------------------------------------------|----------------------------------------------------------------------------------------------------|
| <b>Who is treating me</b>      | Is it you, or somebody I have not met and cannot assess? | The clinicians named, how they are trained, what you sign off and what you do not                  |
| <b>Is it safe here</b>         | I have read the stories, and some of them are recent     | What a real consultation screens for, what gets refused, and what the escalation route actually is |
| <b>Will I look done</b>        | I want to look like myself, only rested                  | Your whole position, applied to Nigerian faces and Nigerian expectations rather than American ones |
| <b>What happens afterwards</b> | Who looks after me when everyone has gone home?          | Aftercare. The single piece of open ground nobody in this market has taken                         |
| <b>Why not just fly</b>        | Istanbul does the whole thing in one price               | The honest comparison, including the parts where they are genuinely better                         |
| <b>What is it like</b>         | What actually happens when I walk in?                    | The consultation, demystified, minute by minute                                                    |

**Open question for the session.** Is there a seventh? The one we keep circling is secrecy, which is a product requirement here in a way it is not in California. Nobody has filmed a surgeon saying out loud how a practice protects a patient's privacy, and we suspect it would land hard. We are not sure whether it is reassuring or whether it makes the whole thing feel furtive. Your call.

### 3. What the rules free us to do

There are four hard constraints. They sound like a cage and they are closer to a brief, because every one of them pushes the content toward the thing that converts anyway.

**No patients, no patient images, no before and after.** MDCN's October 2025 guidance requires explicit written consent for any identifiable patient image, and the American consents behind your existing gallery will not cover Nigerian marketing. The live precedent is a Beverly Hills plastic surgeon who lost a six hundred thousand dollar judgment, upheld on appeal in 2023, for posting patient photographs.

*What that frees us to do:* everybody else in this market sells with results. You cannot, so you sell with judgement instead, which is both more defensible and more like you.

**No brand names.** Botulinum toxin and dermal fillers are prescription-only medicines in Nigeria and may be advertised only in medical journals. Generic names throughout.

*What that frees us to do:* you talk about categories and about what things are for, which is the register of a clinician rather than a vendor.

**Nothing filmed that shows you treating a person in Lagos** until your registration position is in writing. Hands-only, a colleague, or equipment alone.

*What that frees us to do:* it forces the camera onto the conversation, which is the part that actually sells.

**You educate, the clinic books.** Self-advertisement by a practitioner is a disciplinary offence under Part F. Every call to action belongs to MedLYFE.

*What that frees us to do:* you never have to do the thing you have said publicly you will not do.



One more, commercial rather than legal: **no prices on camera**. They are moving, and a figure said out loud in October is wrong by January and lives on the internet for ever. Talk about what the money is for, never what it is.

### 4. Provocations

The brainstorm section. Some of these are good and some are bad, and we would rather be told which than guess. They are roughly ordered by how much we believe in them.

**The second opinion.** Revision work is where your training is deepest and where Lagos is least served. A series answering "I have had something done and I am unhappy" would reach people who are already in the market, already spending, and currently have nowhere credible to go. It is also the most delicate thing on this list, because it is adjacent to criticising other surgeons, which the Council does not permit. Can it be done generously?

**Questions from the hundred and seventy.** There is a real list of people who enquired in March and went quiet. We can anonymise what they actually asked and you can answer the ten most common on camera. Not invented FAQs. Their words. We think this is the strongest idea here.

**What the money is for.** Not prices. The anatomy of why a proper operation costs what it costs: the anaesthetist, the time, the theatre, the follow-up, the thing that is included that nobody counts. This is the piece that makes a cheaper quote look like a worse one without ever naming a competitor.

**The things I have stopped doing.** You have been in practice long enough to have changed your mind. A piece about what you believed ten years ago and no longer believe would be unusually honest and almost nobody would copy it.

**Ageing, as our mothers did it.** How a Nigerian woman of your mother's generation aged, what was expected of her, and what has changed. This is cultural rather than clinical and it is the one piece on this list that could travel well beyond the buying audience.

**The language problem.** Patients here ask for things in a vocabulary nobody writes down: snatched, soft life, body goals. Translating between that and what is clinically possible is useful, funny if you want it to be, and completely unoccupied.

**What I would want, if I were the patient.** The consultation you would ask for, the questions you would ask, and the answers that would make you walk out.

**Who taught you to say no.** The least commercial idea here and possibly the best one. Every surgeon has a moment where they learned to refuse. Yours would explain the whole practice in three minutes.

**The ones we are least sure about, offered so you can kill them.** Reacting on camera to Nigerian cosmetic surgery content, which we suspect is beneath you and cuts against your anti-influencer position. A piece on what you tell people who return from abroad with a complication, which is valuable but risks reading as fear-selling. And anything involving your family, which we have deliberately kept off this list entirely unless you raise it.

### 5. How you want to work on the day

A real choice, and it changes the schedule. Tell us which.

|                    | Scripted to a prompter              | Interviewed                 |
|--------------------|-------------------------------------|-----------------------------|
| What it feels like | Controlled, precise, nothing wasted | Looser, warmer, more of you |



|                       | Scripted to a prompter              | Interviewed                                            |
|-----------------------|-------------------------------------|--------------------------------------------------------|
| <b>What it yields</b> | More pieces per hour, cleaner edits | Fewer pieces, better moments                           |
| <b>Risk</b>           | Can read as performed               | Rambles unless the interviewer is good                 |
| <b>Our instinct</b>   | For the short explainer pieces      | For every long-form piece, and for all of section four |

We think the answer is both, split by setting, which is how the day below is built. But if you hate prompters, say so now rather than at eight in the morning.

**Second open question.** Should anyone be on camera with you? We think yes, for one block: you and the clinic's lead clinician, together. It answers "who is actually treating me" far better than you can alone, and it visibly transfers authority to the person who will be in the room when you are not. Itunu, this is partly a question for you, since it means committing a clinician for two hours.

## 6. Three sites, five settings

Chosen so a viewer can tell which kind of piece they are watching in two seconds with the sound off.

| Setting                         | Site                  | The look                                                              | What it carries                                                                                   |
|---------------------------------|-----------------------|-----------------------------------------------------------------------|---------------------------------------------------------------------------------------------------|
| <b>1. The consultation room</b> | The clinic            | Seated, as if across from a patient.<br>Daylight, real room, unstyled | The highest intent material. What happens at a consultation, the screening, the questions you ask |
| <b>2. The treatment room</b>    | The clinic            | Standing, clinical, equipment visible                                 | Procedure explainers and the safety architecture.<br>No patient, ever                             |
| <b>3. The dark portrait set</b> | Studio                | Your existing portrait look.<br>Near-black, single source             | The position pieces. Refusals, beliefs, what you will not do                                      |
| <b>4. The warm interior</b>     | House or members club | Soft daylight, seated low, no desk                                    | Long-form conversation, the two-hander, and all of section four                                   |
| <b>5. The Lagos exterior</b>    | Same site as four     | Identifiably Lagos without being a postcard. Golden hour              | The "I am here" pieces and the b-roll that makes the library feel local                           |

Settings one and two share a site, four and five share a site. Three moves across the day, which is what makes eight hours honest rather than optimistic.

**Alternative worth considering.** Setting five could instead be Capital Club on the evening of the twenty-first, shot around the event. It would cost nothing extra in crew, it would give the library footage of a real room responding to you, and it ties the content to the evening. Against it: you will be working that night and it is the wrong time to ask you to perform twice.

## 7. The shot list

Longer than the day on purpose. Mark it up and cut it with us.

### Setting one: the consultation room, around 2 hours

The most valuable block. These are what a coordinator sends to one named person who enquired and went quiet.



- **What actually happens at a consultation.** Long form, the whole thing, intake form to written plan
- The questions you ask before you will discuss any procedure at all
- Why the form has to be done beforehand rather than in the room
- What you are looking at while she is talking
- Why you ask about nicotine, and what the answer changes
- Why you ask whether her weight is still moving
- What a written plan contains, and what it deliberately leaves out
- Why you start with "what would you like to change" rather than with a menu
- What happens if you think she should not have it done
- What happens if she changes her mind afterwards
- Whether to bring somebody with her
- What to do with a question that occurs to her after she has left

### Setting two: the treatment room, around 1 hour 30

Generic names only. Hands and equipment, no faces.

- **The safety architecture.** Long form. Who is in the room, what is on the trolley, what happens when something goes wrong
- What is actually in a consent conversation, and why it takes as long as it does
- Botulinum toxin: what it does, who it suits, who it does not
- Dermal filler: the same, plus what you will not fill and why
- Bio-remodelling and skin boosters, including where the evidence stops
- Regenerative treatments: what is supported and what is being sold ahead of the science
- What complication management means in practice
- Why you will not treat on the same day as the consultation

### Setting three: the dark portrait set, around 1 hour 30

Your position pieces. Fastest to shoot because they are closest to what you already say.

- **The refusal list.** Long form. What you will not do, and why the list matters more than the menu
- Looking rested is not the same as looking younger
- The work nobody can point at
- Why ageing is not the problem people think it is
- What "natural" actually means, and why everybody claims it
- Darker skin is not lighter skin with more pigment
- Keloids and scarring, and what they change about a plan
- The most overdone procedure right now
- Why you do not want anybody needing you twice a year
- What it means that you train the people who treat her

### Setting four: the warm interior, around 2 hours

Conversational. The two-hander and the brainstorm pieces live here.

- **Who treats you when she is not in the country.** Long form, two-hander with the lead clinician



- **Aftercare: the part nobody quotes for.** Long form, and on our reading the strongest single piece in this shoot
- **Questions from the hundred and seventy.** Long form, her real questions answered
- Who is responsible for your recovery, by name
- What happens at two in the morning
- What is in the quote, and what arrives as a separate bill later
- Why recovery costs more than people plan for
- What a realistic first fortnight looks like
- When to call somebody, and when not to worry
- Bringing a mother or a sister into the recovery plan
- What you wish patients asked before rather than after
- Whichever three of section four survive the session

### Setting five: the Lagos exterior, around 1 hour

- **Why do this in Lagos rather than fly.** Long form, honest, including what Istanbul genuinely does well
- What a flight home does not come with
- Why you are building here
- What you were told would not be possible in Lagos
- Arrival and walking b-roll, for cutdowns and for the invitation film
- Silent portraits, stills and motion, for the page and the invitations

## 8. Questions only you can answer

Bring answers to these and the scripts write themselves. We cannot write them for you.

- What is the question you are asked most often that you least enjoy answering, and what is the honest answer?
- What do Nigerian patients ask that American patients never ask?
- What did you believe ten years ago that you no longer believe?
- Is there a procedure you have stopped offering? Why?
- What makes you walk away from a case?
- Who taught you to say no?
- What is the thing about Nigerian skin, or Nigerian bodies, that you wish every GP in Lagos understood?
- What is the most expensive mistake a patient can make before she ever reaches you?
- When somebody says they want to look natural, what do they usually actually mean?
- What is the kindest thing you have ever had to tell somebody?

## 9. What eight hours actually yields

|                         | Count                   | Length                   |
|-------------------------|-------------------------|--------------------------|
| Long-form conversations | 6 to 8                  | Four to eight minutes    |
| Short pieces            | 35 to 45                | Thirty to ninety seconds |
| B-roll                  | Enough to cut all of it |                          |



Around fifty publishable assets. At three a week that is four months of publishing, which carries the practice through the evening on the twenty-first, through December when the diaspora arrives, and into January.

**It only holds if the scripts are settled before the day.** A shoot where the script is being argued on set yields about a third of this. That is the single largest risk here, and it is a diary problem rather than a creative one.

## 10. The day

|       |                                          |                                                                      |
|-------|------------------------------------------|----------------------------------------------------------------------|
| 08:00 | Crew in at the clinic, light setting one |                                                                      |
| 08:45 | <b>Consultation room</b>                 | Long form first, while you are freshest                              |
| 10:45 | Relight                                  |                                                                      |
| 11:00 | <b>Treatment room</b>                    | Hands and equipment b-roll last, so you can leave                    |
| 12:30 | Travel and lunch                         |                                                                      |
| 13:30 | <b>Dark portrait set</b>                 | Your own look, least setup                                           |
| 15:00 | Relight. Second clinician arrives        |                                                                      |
| 15:15 | <b>Warm interior</b>                     | Aftercare first in this block. It is the piece we least want to lose |
| 17:15 | <b>Exterior</b>                          | Golden hour, hard stop when the light goes                           |
| 18:15 | Wrap                                     |                                                                      |

If the day overruns, the two pieces that must survive are **what actually happens at a consultation** and **aftercare**. Everything else can be picked up on a half day.

## 11. What has to exist before the day

|                                                                               | Who                  |
|-------------------------------------------------------------------------------|----------------------|
| This session held, and the shot list cut to the day                           | All of us, this week |
| Every script written and approved, compliance applied before Chinwe sees them | CFA, one week out    |
| Prompter confirmed, and the long-form pieces rehearsed                        | Crew and Chinwe      |
| The second clinician committed for two hours and briefed                      | Itunu                |
| Written confirmation that nothing filmed shows treatment of a patient         | The clinic           |
| Three wardrobe changes, one per site                                          | Chinwe               |
| A named owner for the publishing calendar                                     | To assign            |

## 12. Afterwards

The library is worth nothing on a hard drive, and this is where most shoots quietly fail.



**Publish on a calendar, not on inspiration.** Three a week, scheduled, for four months.

**The coordinator sends these one to one.** That is the whole point of bottom-of-funnel content. Somebody enquires, goes quiet, and receives one video that answers the specific thing she hesitated over. That is worth more than any number of public views, and it is the mechanism that fixes what happened in March.

**Cut every long-form piece into its own shorts.** Six long pieces will yield another twenty or thirty cutdowns, which roughly doubles the library for the cost of an editor rather than a second shoot day.

**Keep the masters.** Edition two of Ageless, and every visit after it, inherits this footage. The portrait library, the b-roll and the evergreen explainers should still be working in two years.

## Appendix: generated video stubs

Reusable prompts for generated b-roll, so the library can be extended later without booking a crew. Written to be model-agnostic and self-contained, so they still work whichever tool is in favour next year.

### The two hard lines

**Nothing generated may depict a person who could be mistaken for a patient, a clinician, or Dr Kpaduwa.** Not a face, not a body, not hands performing a procedure. A synthetic patient in medical marketing is a fabricated record, and a synthetic result is a fabricated outcome. That is deceptive on its own terms, and in Nigeria it also walks into ARCON's requirement that claims be substantiable on demand. Every human being in this library is real, consented and filmed.

**Nothing generated may depict an outcome.** No skin smoothing, no body changing, no before becoming after, however abstract the treatment. The moment a generated image implies a result, it is an unsubstantiated claim.

What generated video is genuinely good for here is everything that is not a person and not a result: texture, light, pattern, abstraction, title beds and atmosphere. That is perhaps a quarter of the cutting material in a library like this, and it is the quarter that is tedious and expensive to film.

### How to use a stub

Each one gives the subject, the movement, the light, the palette and the length. Add your own aspect ratio at the end: 9:16 for Status and Reels, 4:5 for feed, 16:9 for the long-form beds. Keep every clip between four and ten seconds, because they are cutaways rather than scenes, and generate three variations of each so the editor has a choice.

Palettes, so the library stays coherent:

**Medlyfe** deep forest green #1F3A2E, chartreuse #C4D7A6, white.

**Lyfe Plastics** near-black #15161A, bronze #A87B4F, warm paper #FAF7F2.

### Title and quote beds

**S01 Ink in water, bronze.** A single thread of bronze pigment entering still, near-black water from the top of frame, unfurling slowly. Macro, shallow depth, one soft key from the left. No vessel edges visible. Six seconds, slow motion, loopable.

**S02 Ink in water, chartreuse.** As S01 but pale chartreuse pigment into deep forest green water. Six seconds.

**S03 Silk settling.** Heavy warm-white silk falling and settling onto an unseen surface, backlit so the weave glows at the edges. Extreme slow motion. No hands, no figure. Eight seconds.



**S04 Dust in a shaft of light.** Fine dust drifting through a single hard shaft of daylight in a dark room. Near-black ground, warm light. Static camera. Ten seconds, loopable.

**S05 Gold leaf fracture.** A thin sheet of bronze leaf breaking apart slowly against near-black, pieces drifting rather than falling. Macro. Six seconds.

### Texture and material

**S06 Raw silk, macro drift.** Slow lateral drift across undyed raw silk, warm paper tone, side light raking across the weave. No pattern, no logo. Eight seconds.

**S07 Clay surface.** Slow push into a matte unfired clay surface, warm neutral, soft top light, fine grain visible. Eight seconds.

**S08 Water surface, dark.** Still dark water, a single drop landing off centre, rings expanding and settling. Overhead, near-black with one chartreuse rim light. Seven seconds, loopable.

**S09 Stone and shadow.** Slow pan across pale limestone in low raking sun, deep shadow on one side. No horizon, no sky. Eight seconds.

### Abstract biology, for explainer beds only

These sit under narration about how tissue behaves. They must stay abstract enough that nobody could read them as a scan, a diagram of a named procedure, or a claim.

**S10 Collagen lattice.** An abstract three-dimensional lattice of fine translucent fibres, slowly reorganising and tightening, warm white against near-black. Not labelled, not anatomical, no body part visible. Eight seconds.

**S11 Lattice loosening.** As S10 but the lattice slowly loosening and separating. Eight seconds. *Use with care: pair it with narration about ageing generally, never with a treatment claim.*

**S12 Cellular drift.** Soft translucent spheres drifting and gently colliding in dark fluid, shallow focus, one chartreuse rim light. Abstract, not a micrograph. Seven seconds.

**S13 Branching flow.** A slow branching flow of pale light through darkness, like a delta from above, suggesting circulation without depicting it. Eight seconds.

### Atmosphere, Lagos

No recognisable faces in any of these. If the model produces a face, discard and regenerate.

**S14 Lagos from height, dusk.** A high wide view of a dense coastal city at blue hour, warm window light beginning, lagoon visible, no landmark that fixes it to another city. Slow drift. Ten seconds.

**S15 Palm shadow on a wall.** Hard afternoon sun throwing palm frond shadow across a pale rendered wall, shadow moving slowly in a light wind. Static. Ten seconds, loopable.

**S16 Rain on glass, warm interior.** Heavy tropical rain on a window seen from a warm lit interior, lights beyond out of focus. No figure. Eight seconds.

**S17 Harmattan light.** A still interior with diffuse dusty golden light through a large window, dust visible in the air, no furniture in focus. Eight seconds.

### Transition and punctuation

**S18 Light sweep, bronze.** A single soft bronze light sweeping left to right across near-black, as a wipe between sections. Two seconds.

**S19 Light sweep, chartreuse.** As S18 in chartreuse over deep forest green. Two seconds.



**S20 Iris of light.** A soft circle of warm light opening from the centre of a black frame and filling it. Two seconds, for cutting into a title.

### A note on honesty

None of these needs to be disclosed as generated, because none of them depicts anything that could be mistaken for a fact about a patient, a result or a clinic. They are texture.

The moment that stops being true, the calculation changes entirely. If anyone later wants generated footage of a room, a building, a piece of equipment or a person, that is a different conversation, and the default answer should be to film it or not use it. In a market where the public's main fear is being deceived by a clinic, a synthetic clinic is a poor place to economise.